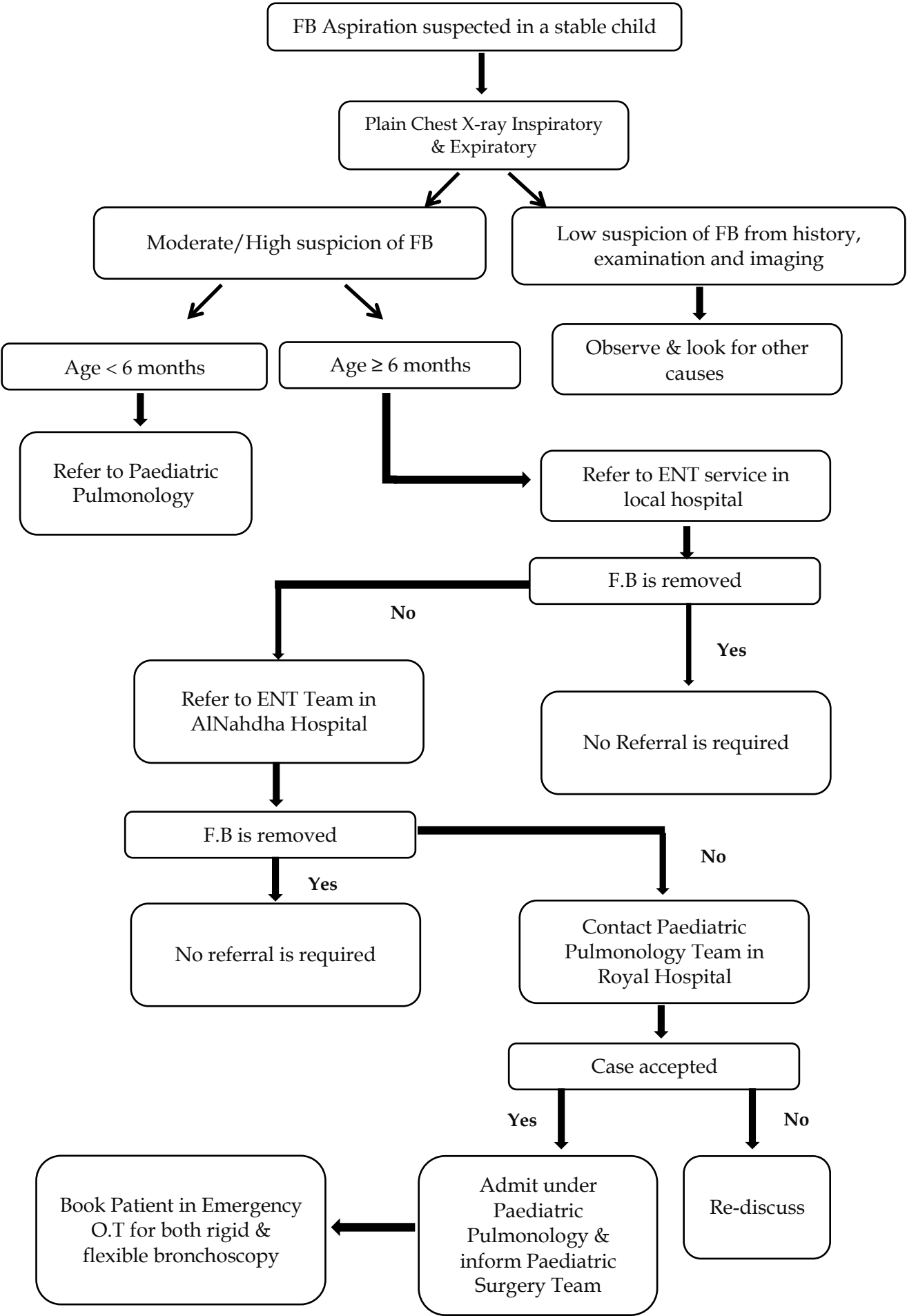


Management Pathway for Foreign Body Aspiration in Children

- Foreign Body (F.B) aspiration in children remains a significant cause of morbidity and mortality. Large F.Bs in main airways requires immediate intervention.
 - In stable patients in whom the F.B is suspected or confirmed, the following protocol should help pediatricians and general practitioners in managing and properly referring children's with F.B aspiration suspected or confirmed.
 - This agreement and management plan was made by professions involved in the management of foreign body inhalation in Oman represented by; Ear, Nose, and throat (ENT) surgeons from Al Nahdha Hospital, Paediatric Surgeons from Royal Hospital, and paediatric pulmonologists from Royal Hospital.
 - This Protocol/ Guideline is applied on all pediatric patients suspected or confirmed foreign body inhalation seen in MOH health care institutes.
- It is agreed among all services that all pediatric patients who are suspected to have foreign body inhalation should have the following:
- 1) If clinically suspected (history and physical examination) to do plain chest x-ray (inspiratory & expiratory). Chest CT scan is rarely indicated.
 - 2) In case of low suspicion of FB from history, examination and imaging to observe the child and look for other causes for his condition.
 - 3) In case of moderate to high suspicion to refer directly to their local ENT service at their hospitals or regional hospitals.
 - 4) In case of refusal or inability of the local/regional ENT service to manage the case or provide the needed service, it should be referred to ENT department in Al Nahdha Hospital where the foreign body can be removed by rigid bronchoscopy procedure.
 - 5) In case of difficulty or failure in extracting and removing the F.B, the ENT consultant on service in ENT department at Al Nahdha hospital can liaise with paediatric pulmonologist oncall to transfer the case under his care at Royal hospital.
 - 6) Paediatric surgeon oncall should be informed immediately about the patient's transfer and care before initiating the transfer.
 - 7) Paediatric surgeon on service should discuss with paediatric pulmonologist regarding the proper plan for the patient and whether rigid bronchoscopy can be re-tried (if done by ENT already) or be done (if not tried by ENT).
 - 8) In cases where rigid bronchoscopy was done by ENT surgeons or paediatric surgeons and failed, or doing it is difficult, then Paediatric Pulmonologists should go ahead with flexible bronchoscopy procedure at paediatric surgeon's presence in the operation theater (**Emergency OT**). Both should be ready for flexible and/or rigid bronchoscopy.

Foreign Body (FB) Aspiration in lower airways Protocol - Algorithm



CT scan is not routinely indicated